

--SUMMARY--

Decision No. 1689/14

20-Oct-2014

L.Petrykowski - M.Christie - G.Carlino

- Continuing entitlement

No Summary Available

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- WSIA

Other Case Reference

- [w4814n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1689/14

BEFORE:

L. Petrykowski : Vice-Chair
M. Christie : Member Representative of Employers
G. Carlino : Member Representative of Workers

HEARING:

September 11, 2014 at Toronto
Oral

DATE OF DECISION:

October 20, 2014

NEUTRAL CITATION:

2014 ONWSIAT 2264

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer decision dated October 11, 2011

APPEARANCES:

For the worker:

K. Stuebing, lawyer

For the employer:

J. Basinger, lawyer

Interpreter:

N/A

REASONS

(i) Introduction to the appeal proceedings

[1] The employer appeals a decision of the Appeals Resolution Officer (“ARO”) of the Workplace Safety and Insurance Board (“Board”) dated October 11, 2011, which re-instated ongoing claim entitlement for the worker, including surgery, allowed a referral for a NEL assessment, and re-instated entitlement to previously rescinded benefit periods. That decision was made after an oral appeal hearing at the Board’s Appeals Branch.

[2] The employer attended the Tribunal hearing with its representative, Mr. Basinger, a lawyer. The worker attended the Tribunal hearing with her representative, Mr. Stuebing, a lawyer.

(ii) Issues

[3] The issues to be decided in this appeal are as follows:

1. Whether the worker has ongoing entitlement to benefits, including for the surgery (February 14, 2008)?
2. Whether the worker has entitlement for a NEL assessment for her right knee?
3. Whether the worker has entitlement for the benefit periods that were re-instated?

(iii) Background

[4] The Panel has reviewed the entirety of the documentary record and the following background facts are noted.

[5] The now 58 year-old worker started work with the accident employer in 1991. On February 24, 2004, the worker was working as a weigh-scale operator and injured her right knee after falling in the employer’s parking lot. The Board’s operating level accepted initial claim entitlement for the worker’s injury. The worker received loss of earning (“LOE”) benefits for various periods of time subsequent to the date of accident and in subsequent years.

[6] In correspondence dated February 1, 2008, the employer advised the Board that the worker’s various recurrences were being attributed to her knee injury, including October 16, 2007, November 19, 2007, November 20, 2007, November 28, 2007, November 29, 2007, and December 11/12, 2007. It was also noted that medical information was submitted for January 22, 2008. The Board Appeals Branch’s decision dated November 7, 2007 noted that various accommodations recommended by the Board’s ergonomist were not yet implemented at the time of the preceding appeal hearing and various periods of lost time due to flare-ups of pain would be compensated for. It was also noted that the recommendations were put into place by January 8, 2008, and the Board ergonomist returned thereafter to review the accommodations and confirmed that the worker’s work was suitable and any risk factors that previously existed would be eliminated. It was further noted that for the October 16, 2007 absence, the worker was not yet aware of the requirement to have medical information to support her absence and payment of benefits was allowed. It was also noted that the worker provided medical information for the November and December periods of lost time and as accommodations were not yet in place, the lost time for those dates was also in order. In regard

to the January 2008 medical information submitted, the accident history was that of a slip and fall at home, and the Board's Claims Adjudicator noted that this was a significant new accident that was not work-related and as such, the lost time was not allowed.

- [7] In further correspondence dated April 2, 2008, the Board's Claims Adjudicator once again reviewed entitlement. It was noted there that the employer had advised of various recurrences including for lost time around February 8 and 11, 2008, and for the surgery dated February 14, 2008. Based on the review of the information on file for February 8 and 11, 2008, the Board's Claims Adjudicator was unable to establish entitlement to LOE benefits. The Board's Claims Adjudicator advised that with the medical information in the record, the surgery of February 14, 2008 was accepted as compensable, and related health care and LOE benefits would also be paid.
- [8] In further correspondence dated July 11, 2008, the Board's Claims Adjudicator once again reviewed entitlement. At that time it was noted that ergonomic changes had been put in place and as the work and the workstation were suitable, the worker would be entitled to LOE benefits if there were objective medical findings to support that her condition had worsened and that the worker was physically unable to perform her work-duties. Based on the review of the information on file at that time, there were no objective medical findings to support that any of the continued absences from work were necessary and as such, there was no entitlement to the lost time for the recurrences.
- [9] In further correspondence dated October 22, 2008, the Board's Claims Adjudicator noted that the worker had claimed additional recurrences on June 26, 2008, July 8, 2008, August 13, 2008, and September 22, 2008. Based on the Board's review of the information on file, there were no objective medical findings to support any of the continued absences from work and as such, lost time for these recurrences was denied. In correspondence dated March 9, 2009, the Board's Case Manager once again reviewed entitlement. It was noted there that a Board Medical Consultant concluded that the right knee strain had fully recovered without evidence of a permanent impairment by August 17, 2005. The Board's Case Manager then concluded that the surgery performed on February 14, 2008 was not related to the workplace fall of February 24, 2004 but rather due to an underlying pre-existing condition, and therefore lost time benefits from February 14, 2008 were rescinded.
- [10] The worker objected to these Board determinations and the matter was referred to the Board's Appeals Branch for further review and consideration. The ARO reversed the Board operating level's determinations, in part, in a decision dated October 11, 2011. The ARO's decision concluded the following:

...ASSESSMENT OF THE EVIDENCE AND SUBMISSIONS

All of the information on the claim record has been carefully considered, particularly as it relates to these issues under dispute.

The worker is a long-term employee who started with the employer 21 years prior as a weigh scale operator at... At the time of the accident in 2004, she was working the 8:00 p.m. to 4:00 a.m. shift and her duties included weighing in and outbound people as well as taking deposits, balancing cash, banking and dealing with customers all day. She testified that most of the time she had a co-worker but did not have one on the night shift in 2004. The most difficult part of her job was getting up and down to get license plates from vehicles. She indicated that before 2004, she had no knee pains or problems and she was able to walk her dogs and play with her grandchildren and gardening and various

other activities. She also denied any pain medication for her knees prior to the February 24, 2004 incident.

She testified that on February 24, 2004, it had snowed and she was going to her car and slipped on ice and twisted her right knee and hit her knee on her car and fell. Her knee was sore at the time and she was swollen by the time she got home and she saw her doctor the next day and was told to elevate her knee and use heat and cold as needed as well as Tylenol. She also had physiotherapy and said it made her knee worse and she indicated that she never had any of these problems pre-accident. She had too much pain and swelling to continue working full-time without missing days after the accident and at the time her husband, daughter and grandchildren lived with her. At the start of her shift, the swelling wasn't too bad but by the end of the shift, her knee would swell to the point where she had trouble walking and driving home. She had trouble concentrating due to pain and had trouble getting up and down in her chair and she could not bend. She also had trouble using the washroom due to changing positions and she said she complained to co-workers and supervisors about swelling and pain and her co-workers became annoyed with her limitations,

She discussed the ergonomic assessment that was done and how a stool was recommended and anti-slip shoes and a new chair but it took over a year to get the chair and the chair did not arrive until 2008. It was also recommended that a camera be installed at inbound and outbound scales so you could see them without getting up, and she indicated that the camera was never put in and the shoes that were supplied were too big for her and made things more difficult.

She indicated that in terms of treatment she underwent cortisone shots and they helped, as well as MRIs and lots of physiotherapy and acupuncture. She also described various medications, including anti-inflammatories, anti-depressants, pain medications, creams as well as three knee braces.

She discussed the [hospital] assessment in 2005 and indicated that she had just come off a shift and went directly to the assessment. She was tired and the assessment lasted a few hours, and her knee was swollen and bruised that day. At [the hospital] she was told that she would fully recover by 2005 but this did not occur and she said, in fact, her knee condition got worse as time went on. She indicated the crutches and canes did not help, and she could only walk for limited periods with a brace.

After the accident, the worker indicated that adjustable desks came in sometime after 2007, however this was a [workplace] change and not just to accommodate her condition. In 2008, she was provided a new chair with arms to help her get up and down but indicated she stood all the time due to her job and this was difficult, as she had difficulty leaning out the window to deal with customers. She indicated that the windows at the transfer station were made for dealing with tractor-trailers and not cars so it was difficult to get paperwork and money from cars as they were lower than the transport trucks that the building was designed for. She indicated she would have slower times and times that were extremely busy and the number of cars that she would deal with would vary per shift. Being at work affected her knee pain and by the end of the shift, she was in considerable pain and had swelling.

She indicated that on March 3, 2006, she was going outside the scale door and the stairs broke and she banged her right knee on the pavement and was taken to hospital. She could not remember if she lost time from work, but indicated that the stairs had been repaired by the employer by the time she worked her next shift.

She was also questioned regarding a November 19, 2007 fall at home, and indicated that she remembered having a few falls and slips at home in that her knee gave out and she fell but she was vague about the details and had difficulty remembering exactly when these falls occurred. She indicated that she lost time in 2008 and said this was for her knee as she had no other medical conditions and said that the lost time in 2008 was

because her doctor told her to stay off work. When she was at home, she stayed quiet on the couch, and said that her job was threatened due to lost time and the union got involved.

Although the worker could not specifically recall the dates of the falls at home, she indicated that her worst fall which was probably late 2007 or early 2008 indicated when she was going down the stairs to her front door and her knee gave out and her one knee went through the stairs.

She was stuck there for approximately 45 minutes until her husband came home and indicated that all of her falls were caused by the knee giving out, and none were due to any other factors and she related all of the slips and falls at home to the original compensable knee injury.

She underwent a total knee replacement in February 2010 and indicated that this was difficult, but her knee is now better than she expected, and after the 2010 surgery, she has been able to do stairs and go upstairs. She also now does a little bit of gardening, can cook, can play with her grandchildren and walk at a reasonable pace with much less pain and swelling and described a big improvement although she does still have some limitations.

She described the February 14, 2008 surgery as an arthroscopic procedure, and indicated that her knee bled for three days following that surgery.

In reaching a decision in this case, I have considered all of the information on the claim record as well as the medical reporting provided and the arguments put forward by the workplace parties.

The worker provided testimony in a credible and straightforward manner and she is a long-term employee who had no health problems pre-accident. She described all of her slips and falls occurring since 2004 being from the original knee injury of February 2004, and indicated that prior to the February 2004 incident at work, she had no circumstances whatsoever where her knee gave out nor did she slip or fall. The medical information indicates a possible pre-existing condition, and it would appear that the incident of February 2004 may have permanently aggravated an asymptomatic pre-existing condition that never did return to the pre-accident state. The worker's family doctor has provided an opinion that the worker did not recover from her knee condition from the date of her injury until the time of the February 2008 surgery, and also indicated that her job duties may affect her knee condition, and indicated that the worker had suffered falls at her house which could be a direct result of her right knee instability. Considering all of the information on the claim record, I am satisfied that although the worker may have had pre-existing pathology prior to the accident of February 24, 2004, this was not symptomatic until the workplace injury and her knee never did return to its pre-accident state. I am also satisfied given the testimony as well as the medical information provided on the claim record that the subsequent surgeries were also directly attributable to the February 2004 workplace accident and as such, the rescinded entitlement should be restored.

The worker is also requesting entitlement to a NEL benefit on an organic basis and given that the worker had no problem pre-accident, but the medical information on the claim record supports ongoing problems since the time of the accident that never did resolve, the worker is to be referred for a NEL assessment to determine whether or not permanent impairment has resulted from the accident of February 2004.

The worker is also claiming entitlement to chronic pain disability, and after considering all of the information on the claim record, it is noted that the worker testified that medication controls her pain and surgery also helped her pain which suggests an organic basis for her pain. The worker has a significant organic problem with the knee which has required surgery and there is also insufficient evidence of marked life disruption, and

under the circumstances, I am satisfied that there is no entitlement to chronic pain disability.

The worker is also claiming entitlement for various recurrences and days of lost time, which includes February 8, 2008; April 23, 2008 to May 5, 2008; May 16 to 27, 2008; June 17, 2008; June 26, 2008; July 6, 2008; August 13, 2008; September 22, 2008 and February 6 to 9, 2009. Since I am accepting ongoing entitlement in this claim, the previously accepted recurrences should be reinstated and additional days being claimed should be adjudicated by the operating area in accordance with medical coverage.

CONCLUSION

1. The appeals resolution officer grants ongoing entitlement, including surgery, and benefits are to be reinstated.
2. The worker is to be referred for a NEL assessment on an organic basis to determine if she has been left with a permanent impairment as a result of the February 2004 accident.
3. Entitlement to chronic pain disability is denied.
4. As stated in the summary, the recurrences and sporadic days of lost time that were previously rescinded are to be reinstated and the additional days should be adjudicated by the operating area according to medical coverage on the claim record.

The objection is granted in part.

[11] Following the release of the ARO's decision, the Board's operating level noted in decision correspondence dated December 14, 2011 that the implementation of that decision involved reinstating benefits for the period February 15, 2008 to April 16, 2008 (full LOE benefits) and April 16, 2008 to August 21, 2008 (partial LOE benefits), as those were the previously rescinded benefits. The Board's operating level also made separate initial determinations for LOE benefits for various days in 2007, 2008 and 2009 as noted in the above-noted implementation letter.

[12] The correctness of the ARO's determinations, as set out in the above-noted issue agenda, is the issue for adjudication in the present Tribunal appeal.

(iv) Law and policy

[13] Since the worker was injured as a result of an accident sustained on February 24, 2004, the *Workplace Safety and Insurance Act, 1997* (the "WSIA") is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[14] Subsection 13(1) of the WSIA states "a worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan".

[15] Subsection 2(1) of the WSIA defines "impairment" as a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[16] Subsection 2(1) of the WSIA defines "permanent impairment" as an impairment that continues to exist after a worker reaches maximum medical recovery.

[17] Subsection 46(1) of the WSIA provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[18] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages: 31, 36, 37, 61, 107, and 300 - Revision #8, would apply to the subject matter of this appeal.

[19] The Panel has considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual* (“OPM”) Document #18-05-03, “Determining the Degree of Permanent Impairment”;
- OPM Document #11-01-05, “Determining Maximum Medical Recovery”;
- OPM Document #11-01-15, “Aggravation Basis”;
- OPM Document #15-03-01, “Recurrences”; and
- OPM Document #18-03-02, “Payment of LOE Benefits”.

[20] The Panel has considered that OPM Document #18-05-03, entitled “Determining the Degree of Permanent Impairment”, states in part:

Policy

Workers who have a work-related permanent impairment are eligible for non-economic loss (NEL) benefits...

To rate permanent impairments, the WSIB uses a prescribed rating schedule, all relevant health care information in the claim file and, if required, the report from a NEL medical assessment...

Permanent impairment

Permanent impairment means any permanent physical or functional abnormality or loss (including disfigurement) which results from an injury, and any psychological damage arising from the abnormality or loss...

(v) The testimony of the worker

[21] The worker testified that she is 58 years old. She started with the accident employer 24 years ago. She was a weigh-scale operator at a transfer station. She had a variety of shifts with various start-times ranging from 3:30 am to 10:30 am. She was on the midnight shift in 2004. Her duties involved weighing trucks and vehicles, taking deposits and payments, going to the scales, and balancing cash. The worker was in a big “fish-bowl” around the scale station. It was half the size of the Tribunal hearing room. There were two work-stations in there and it took four to five stairs to get up. There was paved parking apart from this building. The vehicle would come up to the window on both sides. The window was low and she had to stand up to see the license plates. Sometimes she would have to lean out the window and do visual inspections. She did not have assistance for the majority of the time in the past but now does. The worker did inbound and outbound work at the scale and dealt with the public.

[22] The worker stated that prior to 2004 she had no pain in either knee. She was very active and did lots of walking. She previously worked heavy equipment at a transfer station. This work needed good physical condition. She had no difficulty in doing bulldozer work. The accident in February 2004 involved leaving work and slipping on the snowy surface near her car. She grabbed the car door and bashed her right knee. A co-worker saw this happen. She saw Dr. A., a family doctor, the next day. She did not see him before. The doctor thought it was a bad sprain,

prescribed medication, and ordered an x-ray. She could not continue working as her knee was bad. She had no problems prior to 2004. She had difficulty with steps and slept on a couch at the main level of her house for six years following the accident until her 2010 total knee replacement.

[23] The worker returned to work in 2004 and her job was not changed. She did not fully recover. She took a lot of pills because of the pain. The company pressured her and her partners at work were annoyed by her. She used no medications prior to 2004. The new medications then were Pennsaid, Tylenol #3, and anti-inflammatories. She returned to the weigh scale work to the best of her abilities. She could not go back and forth on her own any more. She had to stand, not sit, to prevent aggravation. The physiotherapist said she had to stand more to lessen the pressure. She remembers that a Board ergonomist came to the workplace. The sit/stand stool was not provided in 2005. There was no anti-fatigue mat. All the scale heights were adjusted at a later time. There were no immediate changes. She did as much as she could in 2004 and 2005 until she could not continue. She then sought medical attention and had cortisone shots to return to work. She missed a couple of days or a couple of weeks, as this varied. This was due to pain and swelling in the knee. She thought there may have been a hospital assessment in 2005. She cannot remember who she talked to there.

[24] The worker stated that her pain did not go away. She had a sharp knee pain and her knee was giving out. Her balance was affected. She had a problem with her ankle. Her walking problem got worse. There was constant swelling from 2005 and heat/ice were used every day. She took anti-inflammatories. She could not do anything. She had two grandchildren, a daughter, a husband, and three dogs in 2004. She did lots of housework, and did stuff in the garden, with her grandchildren, and walked the dogs before the accident. Everything changed after the injury. She could not stand, cook, walk the dogs, sleep in her bed, and be involved with her grandchildren. She could only drive short distances of 4 – 8 km. Her workplace was 8 km away. She was driving very little then and sometimes got a ride to work from a family member.

[25] With respect to the issue of lost time in 2005 and medical notes, the worker testified that the employer allowed for three working days for sickness. She did not know in the past that medical notes were required for these absences. She now brings a medical note. If she was off work for one, two or three days, she did not have to provide medical notes. She missed time in 2005, 2006 and 2007 because of knee problems and related swelling. She was okay at the beginning of her work-shift but she could not walk by the end of the shift. The pain was 11 on a scale from 1 to 10 by the end of her work-shift. She would sit for short periods of time at work, and sometimes this was more uncomfortable. Her chair at work was eventually replaced. She still had to stand and lean out the window at work and did this with difficulty. She was not better by 2007.

[26] With respect to a November 2007 incident, the worker stated she was going down from the main level of her house and her knee “gave way”, and went through the rail of the stairs. She was stuck there for an hour and a half. She had some warning, in the beginning, when her knee was about to give out. She later did not have such warning. There was no warning at the time of the November 2007 incident. This symptom started within six months of the date of accident and it was not a new problem by 2007. The worker stated there was another fall at the transfer station scale in January 2008. The second step on the stairs broke and she went flying. She had a coffee in her hand and twisted her knee.

[27] The worker then saw Dr. Ali and had surgery. This made her condition worse. It bled for three days. This did not help things. She thought the surgery would help her knee but it did not. She later received cortisone medication. She missed time from work due to her knee in 2008 and 2009, and provided medical notes on that occasion. She went for physiotherapy and the first two places did not help. She then went to a rehabilitation institute which helped her “build things up”. She could not have gone to work for the missed days between 2005 and 2009. In retrospect, she now understands it would have been better to stay off work but she felt her job was threatened at the time. This was not initially covered by the WSIB and she had to use up her sick days. The employer took away her sick days. She had gained these sick days and had a bank of them in 2004.

[28] The worker stated she had total knee replacement surgery in 2010. It was scary for her at first but now she does much more. She was off work for six months. She has not been doing bad since the therapy. With respect to the documentary reference about falling at home, she said “anything is possible” and stated that if the doctor said it, it is what happened. There is no lost time now and she does her full-time job in the weigh-scale. Her husband is on medical leave.

[29] The worker stated she has a hard time remembering dates and it is possible the dates could be mixed up. She confirmed that she slipped at work. She slipped at work in January 2008 because of the stairs. She is definitely sure that her second slip was at work. She does not remember the home incident date. The second work incident is marked in the record. She could drive home following her initial workplace accident. She was working 8:00 pm to 4:00 am then. This accident was the morning of the 24th day of the month. She guesses that she waited a full day before seeing her doctor after the date of accident. She did not have a family doctor at the time and went to see a doctor close to home. Her prior doctor retired.

[30] The worker stated that there was no pre-existing knee problem prior to the accident in 2004. She cannot remember the date she returned to work. She went back to her weigh clerk duties on a full-time basis but did them slower. She could be doing a lot of walking there. There was lots of squatting and bending to pick up boxes. It was different than it is now. All the paperwork was on the floor back then. It would take two to twenty minutes per fax. The ergonomist came to see her at workplace. She cannot recall the March 21, 2005 return-to-work visit. She agrees that the position required walking and standing more than half the time. There was not a lot of sitting. The knee-locking was a problem six months following the date of accident. Dr. Moran was the specialist and the head of the emergency department at the hospital. She saw him at his own practice when she twisted her knee.

(vi) The submissions of the employer’s representative

[31] On behalf of the employer, Mr. Basinger submitted it was twenty years since the date of accident and the worker’s memory was fuzzy. The medical evidence needs to be closely referred to. The family doctor initially diagnosed a knee sprain and the worker was not off work for very long. She was referred to an orthopedic specialist who noted the worker had a strain and an MRI was arranged. There was a second incident involving stairs in May 2004. The MRI was unremarkable except for chondromalacia. The worker was assessed at the REC and was seen by the orthopedic specialist, Dr. Urovitz. The worker’s physician was an advocate for the worker as suggested by his December 1, 2004 letter.

[32] Mr. Basinger also noted that there was a two-year medical gap between August 30, 2005 and November 2007. The worker had a different knee by that time from a medical perspective and the knee locked up, which was a new complaint. The new MRI from January 13, 2008 suggested chondromalacia with degenerative cysts. This was a more deteriorated knee and the contemporaneous medical reports suggested a degenerative/osteoarthritic diagnosis. The worker slipped and fell at home on January 22, 2008 and the worker's testimony was fuzzy about the home and work incident dates. Dr. Ali could not say yes to the question about whether the surgery was related to the 2004 workplace accident in his report dated February 28, 2008. Dr. Hamami was acting as an advocate for the worker in his report dated March 24, 2008. The x-ray in November 2008 showed bilateral degenerative changes and Dr. Ho's opinion did not support the knee problem being related to 2004. There was a lack of continuity and the new meniscus problem was not related to the past strain. The ARO's decision should be reversed.

(vii) The submissions of the worker's representative

[33] On behalf of the worker, Mr. Stuebing submitted that there was clear reporting of the worker's functional impairment following 2004/2005, including significant restrictions. The MRI suggested there could be post-traumatic changes. Dr. Hamami did not feel that the chondromalacia would cause severe pain, meaning that the underlying conditions were not responsible for the worker's ongoing knee problems. The REC confirmed a functional intolerance and had a variable prognosis. The worker's condition did not resolve subsequent to 2004. The knee clinic should not be given the same weight as the family doctor's reporting. Dr. Hamami's letter is not advocating for the worker. The worker testified that she greatly cooperated with her work to her detriment. The medical documentation suggests an ongoing problem. The worker testified her symptoms were ongoing and the ARO's previous decision noted that continuity was established over the relevant period. Dr. Ali stated the worker's knee pain was "chronic" as of November 2007. The worker's letter to the WSIB on file fits with continuity being established. Dr. Ali's report supports that the accident made a significant contribution to the worker's knee condition. The family doctor supports this was all the same condition. The recurrences in 2008 and 2009 are medically supported. The ARO got the decision right. The 2004 accident is linked to the tears as a significant contributing factor. There was a permanent impairment according to the MMR policy from 2004 and the worker rightly received a NEL award. The medical evidence should be weighed in the worker's favour, and the benefit of the doubt should go in the worker's favour if the evidence is equal. The worker was credible at the hearing. Dr. Ali's report supports the worker's appeal and a causal link. The employer's appeal should be denied.

(viii) Analysis and conclusions

1. Whether the worker has ongoing entitlement to benefits, including for the surgery (February 14, 2008)?

[34] The Panel has firstly assessed the issue of whether the worker should have ongoing entitlement to benefits, including for the surgery (February 14, 2008) on her right knee. The ARO granted such claim entitlement and after having considered the totality of evidence presented in this appeal including the documentary record and the testimony from the worker, the Panel confirms the decision to grant ongoing entitlement to benefits for the worker's

compensable right knee injury. As such, the employer's appeal is denied in this respect for the following reasons.

[35] The Panel has considered that OPM Document #15-03-01, entitled "Recurrences", states that "to identify a recurrence, the WSIB must confirm that there is clinical compatibility between the original injury or disease and the current condition, or a combination of clinical compatibility and continuity". That Board policy goes on to note that clinical compatibility needs to be established between the worker's current condition and that which followed the initial compensable accident. The Board policy goes on to note that continuity ("a connection between the original clinical condition and the most recent problem or problems") also needs to be considered. The standard of proof required in Tribunal proceedings is proof on the balance of probabilities. For the employer's appeal to be successful, the employer would need to particularly establish on a balance of probabilities that clinical compatibility does not exist between the worker's initial right knee injury (February 24, 2004) and the later surgery (February 14, 2008) involving the worker's right knee.

[36] The Panel has considered that the worker had a compensable right knee injury on February 24, 2004, which was initially diagnosed as a knee strain by a physiotherapist on March 8, 2004. Dr. Hamami, the worker's family doctor, diagnosed a knee sprain on March 15, 2004, and also noted that he first assessed the worker on February 26, 2004 for this workplace injury. The worker's right knee symptomology continued and Dr. Hamami made a referral to an orthopedic specialist on April 26, 2004 for "trauma (Rt) knee 3 months ago". The specialist, Dr. Urovitz, assessed the worker on May 12, 2004 noting that the worker had a strain and recommended conservative treatment. The worker continued with physiotherapy and then had an MRI scan of her right knee on June 23, 2004 noting findings of "post traumatic of the patella however differential would include chondromalacia". The worker's symptomology continued and a right knee strain and effusion was diagnosed on July 26, 2004 by a physiotherapist. Dr. Urovitz noted on August 31, 2004 that the worker's MRI showed "chondromalacia patella" and that the worker should continue with her strengthening exercises. The worker was then referred to the REC where Dr. Marks, an orthopedic specialist, wrote the following on September 28, 2004, in part:

...Diagnosis:

a) Accident - Related

1. Right knee strain.

b) Non Accident – Related

1. Chondromalacia patella (may have been exacerbated by the accident)

Recommendations:

a) Investigations/Consultations:

None recommended at this time.

b) Treatment

This client should pursue 8 weeks of an active physiotherapy for the right knee. This should include range of motion, stretching, and most important, a progressive quadriceps and hamstrings strengthening program and a home exercise routine...

c) Medical Restrictions:

There are no mandated medical restrictions for physical activity.

d) Tolerance/Functional Limitations:

[The worker] may have some tolerance limitations for squatting, kneeling and prolonged standing on a variable basis. We would expect her activity tolerance to improve as she strengthens her right knee musculature.

Prognosis:

Prognosis would be considered variable in this client. Should she continue to have ongoing symptoms, a re-evaluation with a lower extremity specialist in a specialty program may be warranted...

[37] On March 3, 2005, Dr. Hamami, wrote to the Board noting that the worker's "symptoms did not completely resolve. She had problems with knee pain and swelling. No changes in diagnosis". Dr. Hamami also then noted that the worker did not completely recover and had limited knee flexion when she was assessed on February 26, 2005, along with effusion and tenderness. He further confirmed that the diagnosis was still a right knee sprain and advised that the worker will not be able to do the same work load as prior to her injury. Dr. Cameron, an orthopedic specialist, later noted on March 4, 2005 that the worker "sustained a contusion to her right patellofemoral joint and continued anterior knee pain with a feeling of instability" and that her x-rays "showed no evidence of patellofemoral arthritis". He also noted in the summary report of that same date that the worker could return to work with restrictions. The worker continued with physiotherapy following this assessment. The worker was discharged from physiotherapy and a related discharge report dated August 30, 2005 explained the following, in part:

...At the time of her discharge, [the worker] reported that her level of right knee pain was reduced and it had become intermittent, rather than constant, in nature. [The worker] was still prone to occasional pain flare-ups and episodes of swelling in her right knee, however, these episodes were occurring less frequently. Throughout her complex physiotherapy program, [the worker] continued to work full hours and full duties as a weigh scale operator. At the time of her discharge, [the worker] demonstrated functional range of motion in her right knee. She had discontinued wearing her brace on a regular basis, using it only occasionally for difficult evenings at work. At the time of her discharge, [the worker] demonstrated normal reciprocal gait. She reported better tolerance for standing, bending and squatting activities at work, and that she was able to perform all required duties with proper pacing.

[The worker] was discharged with a complete home exercise program, with a recommendation to perform her exercises on a daily basis in order to maintain and to continue to improve her pain control, knee stability and functional tolerances. [The worker] has been advised to follow up with her family physician regarding her knee...

[38] The worker wrote to the Board after receiving a September 8, 2005 letter, noting that she still had knee problems, including that her knee was locking up. Following a gap in medical documentation, the worker complained of right knee pain to her family doctor and swelling as of June 6, 2007, and noted she had missed time from work on account of this. The doctor's clinical records also suggested that the worker suffered from severe right knee pain earlier in 2007, either March 28 or May 28, 2007 as the notation was not completely legible. The worker also attended the Board's Appeals Branch on October 1, 2007 and testified at a hearing that "she has never been pain free since the accident", she takes intermittent days off when her right knee was "really bad" to help cope with her job, she only takes medication and does exercises as physiotherapy and cortisone shots did not help her in the past, and that she has been using her knee brace "on and off". The worker was shortly thereafter assessed by Dr. Ali, an orthopedic specialist, on November 28, 2007, who noted the following, in part:

...Thank you for referring [the worker] for assessment of her chronic right knee pains. This lady has a history for asthma, she takes puffers as well as a nerve pill. She has no known drug allergies. She smokes 1/2 pack per day. She works for the [employer] in the Weigh Station, weighing trucks and heavy materials.

She had an injury in February 2004. She was going to her car in a parking lot at work, she slipped and injured her right knee, twisting it and out of place. She went on to have multiple treatments including various types of braces, several types of physiotherapy, medications and cortisone injection, which were ineffective, as well as having a second opinion with Doctor Urovitz and apparently having a WSIB Assessment. She has been back to work intermittently. Now, she feels like her right knee locks up and that she gives way such that she has swelling on a chronic basis.

X-rays were reviewed showing mild changes of the PF and medial compartment.

Examination is very difficult today as she is very touch sensitive, particularly over the medial aspect, reluctant to bend her knee past 95 and extension 0. There is no instability. No clear findings of meniscus abnormality...

[39]

The worker then received another MRI of her right knee on January 13, 2008, which revealed a medial meniscus posterior horn tear, moderate joint effusion and degenerative chondromalacia of the patella medially, and a potential medial collateral ligament strain. Dr. Hamami completed a Form 8 dated January 23, 2008 noting the worker had a repetitive strain injury to her right knee with osteoarthritis and another Form 8 was completed on that date noting this was a "chronic condition". The worker also experienced a slip and fall injury at home when her knee locked up and she fell around January 22, 2008, which another Form 8 dated January 23, 2008 noted by Dr. Hamami. Dr. Ali assessed the worker on January 30, 2008 noting the MRI findings and recommending arthroscopic surgery for her right knee, which was carried out on February 14, 2008 for a medial meniscal tear, anterior synovitis, and "chondromalacia Grade 2 – 3". Dr. Ali also wrote a report on February 28, 2008 responding to a number of solicited questions, as follows:

1. Please provide your recent treatment dates, diagnosis and any specialists reports in respect to [the worker's] right knee.

November 28, 2007, January 30, 2008, February 14, 2008, February 28, 2008.

Torn medial meniscus, patellofemoral chondromalacia, medial femoral condyle chondromalacia with synovitis of right knee.

2. In your opinion is [the worker's] February 2004 work place accident a significant contributing factor to the above noted diagnosis? Please explain.

I would anticipate that the patient's workplace injury would have exacerbated and may have led to the medial meniscus tear, but I would not expect it to be a specific cause of the changes to the patellofemoral joint and medial femoral condyle. Synovitis can exist in regards to an associated medial meniscus tear.

3. In your opinion, is [the worker's] February 2004 workplace accident a significant contributing factor to your recommendation and performance of the recent right knee arthroscopy?

The arthroscopic surgery that was just carried out was recommended due to her medial meniscus tear.

4. In your professional opinion, does [the worker] have any permanent impairment in her right knee as a result of her February 2004 work place accident?

I do not anticipate any permanent impairment in regard to her right knee due to a medial meniscus tear. Many people have torn medial menisci, have arthroscopic surgery and

have no expected long term permanent impairment. I do not identify this to be happening with [the worker].

[40] Dr. Hamami later wrote a report dated March 24, 2008 noting that in his opinion, the worker “did not recover from her knee condition from the date of injury until January 2008” in reference to the February 2004 workplace accident. He also noted that the worker “suffered a fall at her house which could be a direct result for her (R) knee instability”.

[41] Dr. Hamami’s report dated May 19, 2009 noted that the worker’s “symptoms of right knee pain and locking improved between the dates indicated. Residual joint swelling persisted since February 2004. Note is made of post traumatic changes of the patella in the MRI report dated June 23, 2004...As per Dr. Ali’s report dated February 28, 2008, the initial injury could be a precipitating factor to her condition resulting in medial meniscus tear”. Dr. Hamami’s report dated September 17, 2009 noted that “chondromalacia patella is a condition of overuse of the knee joint. It is a very common cause of knee pain in adults. It is not caused by acute injury and is unlikely to cause the severe pain that [the worker] was presenting with...”. Dr. Ho, a Board Medical Consultant, reviewed the file on March 9, 2009 and concluded that the worker’s “degenerative changes seems to be a natural process unrelated to 24/2/04 [incident]”.

[42] The Panel has also considered the worker’s testimony at the Tribunal hearing, which was found to be forthright, straightforward, and credible. It was largely aligned with the existing information in the documentary record, including the testimony that was summarized from the Board’s Appeals Branch in previous decisions there. In essence, the worker has testified that her right knee problems were ongoing from the date of accident on February 24, 2004, and this changed the way she had to work and live/function outside of work. The Panel accepts this evidence as significant support for the worker’s claim that continuity has been established in this case. The Panel also finds it significant that the worker was missing sporadic/intermittent time from work from 2005 to 2007 due to problems with her right knee, further suggesting no resolution of her right knee symptomology over the material period.

[43] The Panel has carefully considered the extant medical evidence. There is no evidence of substance that the worker fully recovered from her compensable right knee injury. The worker was discharged from physiotherapy around August 2005, but the discharge report did not suggest that the worker had made a full recovery. Rather, that report stated that the worker was having intermittent right knee pain, occasional flare-ups, episodic swelling, occasional use of a knee brace, and better tolerance for physical activities. All of this suggests that the worker was left with a functional and physiological deficiency affecting her right knee with no ultimate recovery being clinically identified by that time or thereafter. The earlier REC report did not count the worker’s prognosis as good. Rather, Dr. Marks noted there that her prognosis was “considered variable” and suggested further specialist involvement would be required if the symptomology did not abate. The worker received such involvement with Dr. Cameron in 2005 who also noted the existence of the worker’s ongoing problems.

[44] By the time of the early half of 2007, Dr. Hamami’s clinical records showed that the worker was having ongoing right knee problems. Dr. Ali noted in his consultation report from November 2007 that these were “chronic” problems, which does not suggest a new or acute knee condition/injury was culpable for the right knee symptomology experienced by the worker. In the Panel’s view, this suggests that following 2005, the worker’s condition was chronic and she worked and lived with it to the best of her ability. According to her testimony at the Tribunal hearing, this necessitated sleeping on a couch on the main-level of her house for many years

following the workplace accident, to avoid stair climbing to the upper-level. The worker also gave up on physiotherapy and other medical modalities, like cortisone shots, as these did not solve her ongoing right knee problems. The worker understood that she was not a surgical candidate in 2004 or 2005 but was only provided a surgical intervention in early 2008 following her right knee symptomology being further investigated in 2007. There is some suggestion that the worker's non-occupational fall led to the right knee symptomology, but the worker had right knee symptomology from the early half of 2007, pre-dating the incidents in late 2007 or early 2008 that were suggested by the employer's representative as being responsible for her right knee problems.

[45] The Panel has also carefully considered the nature of the worker's underlying right knee condition. The Panel has been provided with no medical evidence of substance that the worker suffered from any right knee symptomology prior to the workplace accident on February 24, 2004. Prior to this accident, the right knee did not deleteriously affect the worker at home or at work, but subsequent to that accident, the right knee had a persistent impairing quality on the worker's work and home life. In the Panel's view, in the absence of a clinically significant ongoing underlying condition, this suggests that the workplace accident was a significant contributing factor in the worker's ensuing experience of sporadic and intermittent lost time from work and symptomology. This would include the need for the surgery on February 14, 2008, as well.

[46] While there is some suggestion that the worker's underlying medical condition of knee degeneration and/or chondromalacia was significant to the worker's experience of symptomology in 2007 and 2008, the Panel finds it significant that the worker's initial compensable right knee injury was relatively serious, requiring extensive medical investigation and rehabilitation without ultimate clinical resolution of her right knee symptomology. Dr. Marks, the REC specialist, suggested that the worker's accident-related right knee strain had not resolved by September 28, 2004 and suggested that the worker's chondromalacia "may have been exacerbated by the accident". The Panel finds it significant that Dr. Marks suggested that the compensable workplace injury exacerbated the worker's underlying chondromalacia condition. Dr. Hamami's letter to the Board dated March 3, 2005 made it clear that the worker "did not completely recover". The worker was experiencing instability in her knee in 2005 and noted in her letter to the Board of that same year that she was experiencing a locking phenomenon in her knee. This is also consistent with the worker's testimony at the Tribunal hearing.

[47] The Panel has further considered that there was evidence of meniscal pathology to the worker's right knee identified on the MRI in 2008, but previously from a clinical perspective this was not identified. This includes Dr. Ali's assessment in November 2007, where meniscal pathology was not suggested. The fact that the worker did not have a meniscal injury identified earlier than 2008 does not mean that the workplace accident from February 24, 2004 did not have a significant contributing effect on its development in the worker's right knee. The Panel accepts Dr. Ali's opinion, as a qualified orthopedic surgeon and specialist, on February 28, 2008 that he anticipated "that the patient's workplace injury would have exacerbated and may have led to the medial meniscus tear". This was also later supported/reiterated by Dr. Hamami in his letter dated May 19, 2009. As such, the Panel finds on a balance of probabilities that the worker's workplace accident of February 24, 2004 was a significant contributing factor in the development of the worker's meniscal tear identified in 2008. While other factors including

degeneration may have played a role in the development of this meniscal pathology, the test for entitlement is whether work-related factors played a significant contributing role in the development of the claimed condition. The Panel also accepts that the workplace accident exacerbated the worker's underlying chondromalacia condition, something that was first suggested by Dr. Marks. The Panel also finds it significant that Dr. Hamami stated on September 17, 2009 noted that chondromalacia patella was "unlikely to cause the severe pain that [the worker] was presenting", and this further suggests that since the worker had no pre-accident right knee condition, the workplace accident set in motion a series of events that led to severe and persisting pain on the backdrop of an underlying chondromalacia condition. The chondromalacia could not solely explain the worker's clinical presentation, highlighting the significant impact that work-related factors had in the worker's clinical presentation.

[48] The Panel has considered that the surgery carried out on February 14, 2008 was related to both the chondromalacia and meniscal pathology. The worker's initial compensable injury was serious and traumatic. The worker has established continuity for right knee symptomology from her workplace accident in 2004 to 2008. The Panel accepts on a balance of probabilities that there is clinical compatibility between the workplace accident in 2004 and the need for surgery in 2008. This is not a case where the worker was symptomatic from an underlying knee condition and then went on to have a knee injury that persisted. This is a case where a worker had no pre-existing right knee condition of clinical significance and went on to have a compensable/traumatic right knee injury that persisted in terms of symptomology and functional limitation until such time that corrective surgery was deemed necessary and undertaken in 2008. The Panel finds that the workplace accident was a significant contributing factor in the need for this surgery and therefore the worker has ongoing entitlement for this condition.

[49] As noted earlier, for ongoing entitlement and a recurrence to be allowed in the worker's case, OPM Document #15-03-01 states that "clinical compatibility between the original injury or disease and the current condition" must be established. The Panel finds that this has been established on a balance of probabilities in the worker's case. When coupled with our earlier finding that continuity has been established, the worker's ongoing claim entitlement on a recurrence basis is maintained. Having considered the totality of evidence in this appeal, including the medical and non-medical evidence, the Panel finds that the evidence weighs in the worker's favour on a balance of probabilities that the scope of her entitlement be confirmed. Therefore the employer's appeal to rescind such claim entitlement is denied.

2. Whether the worker has entitlement for a NEL assessment for her right knee?

[50] The Panel must next determine whether the worker should have entitlement for a NEL assessment of her right knee in relation to the compensable injury. In reaching our decision that the worker rightly has entitlement for a NEL assessment for her right knee, the Panel has considered the totality of medical and non-medical evidence before us, including the testimony from the worker in the context of the WSIA and applicable Board policy.

[51] The Panel has considered that the WSIA provides a definition of permanent impairment in subsection 2(1) stating "impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss while "permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery. Subsection 46(1) of the WSIA sets

out entitlement in instances where the worker is found to have a permanent impairment stating “if a worker’s injury results in permanent impairment, the worker is entitled to compensation under this section for his or her non-economic loss”.

[52] The Panel has also considered that Board OPM Document #11-01-05, entitled “Determining Maximum Medical Recovery”, states that:

...Workers reach maximum medical recovery (MMR) when they have reached a plateau in their recovery and it is not likely that there will be any further significant improvement in their medical impairment.

[53] The Panel has further considered that Board OPM Document #11-01-05, sets out factors which are to be considered when determining whether or not a worker has reached MMR, as follows:

Identifying MMR

If a worker has a permanent disability/impairment, decision-makers identify when MMR is reached and record the date. A worker may have reached MMR and still be receiving treatment, such as physiotherapy or drugs where a significant improvement is unlikely.

Decision-makers determine when MMR is reached based on the following information which includes but is not limited to

- clinical reports from the treating health professional(s)
- specialists' report(s), where appropriate
- reports from agency(ies) providing treatment and/or evaluation, (e.g., Regional Evaluation Centres)
- information from the worker on his/her medical impairment
- external, evidence-based medical/scientific guidelines on disease and injury-specific impairment and treatment,
- the opinion of WSIB clinical staff, if obtained.

[54] The Panel notes that the worker’s initial compensable injury stems from February 24, 2004. As noted earlier in this decision, there was no evidence of substance that suggested that the worker had made a full recovery from that injury by 2007 or by the time the surgery was carried out on February 14, 2008. In the Panel’s view, the medical evidence in the documentary record suggests the existence of an ongoing impairment of the worker’s right knee following the workplace accident. The worker had reduced range of motion (flexion) in her right knee to 120 degrees noted on May 12, 2004, 110 degrees as of August 20, 2004, 30 degrees as of February 26, 2005, and 90 degrees as of May 28, 2005. The worker was discharged from physiotherapy around August 2005 without objective information being provided about range of motion. However, by the time she was re-assessed on November 28, 2007, the range of motion was identified to be 95 degrees, not dissimilar to the reduced range of motion finding over two years earlier (90 degrees). All of this suggests that the worker had a permanent impairment in the form of a “physical or functional abnormality or loss” affecting her right knee following her 2004 workplace accident for the purposes of a NEL assessment to be ordered as of November 2007.

[55] However, the worker later underwent corrective surgery (February 14, 2008) which, with post-surgical physiotherapy/rehabilitation, improved the worker’s range of motion findings by January 9, 2009, which means that the worker was not at MMR as of November 2007. The

Board accepted that the worker should receive a NEL assessment following further investigations done after the release of the ARO's decision. The NEL rating was accomplished on October 18, 2012 and provided the worker with a NEL benefit on the basis of a "torn meniscus and/or meniscectomy" and "arthritis due to any etiology, including trauma; chondromalacia" rather than reduced range of motion findings. The Panel finds that both categories are properly recognized by the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised) ("the AMA Guides"), which is the Board's framework for assessing permanent impairment in workers' compensation cases. Given that the Panel has earlier found that the workplace accident was a significant contributing factor in both the meniscal injury (and need for meniscectomy) and the condition of the chondromalacia, the Panel finds on a balance of probabilities that a NEL assessment was properly afforded to the worker following her surgery and following the release of the ARO's decision, which did encompass two correct and compensable categories of impairment under the AMA Guides.

[56] As such, there is no basis to rescind the worker's entitlement for a NEL assessment in relation to her compensable right knee injury and therefore the employer's appeal is denied. Neither of the respective workplace parties that attended the Tribunal hearing for this appeal made any submission about the correctness of the NEL assessment process or the quantum of the NEL award, and the Tribunal makes no findings or ultimate determinations about that issue since it was not specifically before us from a jurisdictional vantage point. The ARO ruled on October 11, 2011 that the worker should be afforded a NEL assessment for her organic-based right knee injury and the Panel finds that there to be no basis to rescind that entitlement for the aforementioned reasons.

3. Whether the worker has entitlement for the benefit periods that were re-instated?

[57] Having earlier recognized that the worker is entitled to ongoing entitlement, including for the surgery carried out on the worker's right knee on February 14, 2008, the Panel further considers the employer's appeal concerning the benefit periods re-instated by the ARO. Having considered the totality of evidence before us, the Panel finds that the benefit periods in question were correctly re-instated and therefore the employer's appeal is also denied in this regard.

[58] As noted earlier in this decision, following the release of the ARO's decision, the Board's operating level noted in decision correspondence dated December 14, 2011 that the implementation of the ARO's decision involved re-instating benefits for the period February 15, 2008 to April 16, 2008 (full LOE benefits) and April 16, 2008 to August 21, 2008 (partial LOE benefits), as those were the previously rescinded benefit periods. The Board's operating level also made separate initial determinations for LOE benefits for various days in 2007, 2008 and 2009 as noted in their implementation letter, and these determinations are not before the Tribunal in the present appeal as there is no final decision of the Board about them within the meaning of subsection 123(1) of the WSIA. Therefore, the Panel is particularly assessing the issues in the instant appeal related to the re-instated benefit periods of February 15, 2008 to April 16, 2008, and April 16, 2008 to August 21, 2008, as above-explained, directly flowing from the ARO's decision.

[59] As it relates to the issue of LOE benefits from February 15, 2008 in this case, OPM Document #18-03-02 entitled "Payment of LOE Benefits" states:

If the nature or seriousness of the injury completely prevents a worker from returning to any type of work, the worker is entitled to full LOE benefits, providing the worker co-operates in health care measures as recommended by the attending health care practitioner and approved by the WSIB. If the worker does not co-operate, the WSIB may reduce or suspend the worker's LOE benefits.

[60] The Board policy indicates that when an injured worker experiences a wage loss as a result of a work-related injury, she is entitled to LOE benefits. This is aligned to subsection 43(1) of the WSIA which states that “a worker who has a loss of earnings as a result of the injury is entitled to payments under this section beginning when the loss of earnings begins....”. According to subsection 43(1)(d) of the WSIA, such payments for LOE benefits cease on “the day on which the worker is no longer impaired as a result of the injury”. In this light, the Panel finds on a balance of probabilities that the worker was totally impaired on a temporary basis following her compensable right knee surgery on February 14, 2008 and therefore entitled to full LOE benefits from that date.

[61] According to Dr. Ali's post-operative report dated February 28, 2008, the worker was “having a lot of pain” and recommended that she not return to work for four weeks, with a plan to further re-assess her in “6 to 8 weeks”. The worker's physiotherapist completed a return to work document dated March 20, 2008 noting that the worker was unable to participate in any work until April 12, 2008. Dr. Hamami's Report of Worker's Function dated April 9, 2008 noted the worker could return to modified duties around April 16, 2008 at half-days (four hours per day). Therefore, the worker had medical authorization to be off work following her surgery until she returned to part-time modified duties on April 16, 2008 (according to the employer's Form 9 dated April 24, 2008). As such, her entitlement to full LOE benefits in the post-operative period is confirmed.

[62] The worker's return to modified duties at half-time hours from April 16, 2008 onwards resulted in a partial (not total) loss of earnings attributable to her compensable surgery and therefore the worker was rightly entitled to partial LOE benefits from April 16, 2008. The Board continued paying the worker LOE benefits at various levels until August 21, 2008, due to the loss of earnings associated with her post-surgical rehabilitation and attempts to re-integrate her into modified work on a graduated basis. These benefits were later rescinded by the Board's operating level and then re-instated by the above-noted ARO's decision. The Panel finds on a balance of probabilities that the worker's experience of loss of earnings over the material periods, February 15, 2008 to April 16, 2008, and April 16, 2008 to August 21, 2008, were a consequence of the numerous difficulties that were medically identified following her compensable right knee surgery. Therefore there is no basis to rescind the worker's claim entitlement over these re-instated benefit periods, particularly since the Panel has earlier determined that the worker had ongoing entitlement under her claim which also specifically included the right knee surgery that was carried out on February 14, 2008. As such, having considered the totality of evidence before us, the Panel has determined that the employer's appeal is also denied in this regard.

DISPOSITION

[63] The employer's appeal is denied.

DATED: October 20, 2014

SIGNED: L. Petrykowski, M. Christie, G. Carlino